CURB-65 — Community-Acquired Pneumonia Severity

adults with CAP · casualty

CURB-65 0-5

CRITERIA (1 POINT EACH)

Item	CURB-65	CRB-65
Confusion (new disorientation in person, place or time)	+1	+1
Urea >7 mmol/L	+1	—
Respiratory rate ≥30/min	+1	+1
BP: systolic <90 or diastolic ≤60 mmHg	+1	+1
Age ≥65 years	+1	+1
Total	**0–5**	**0–4**

CURB-65 — 30-DAY MORTALITY

- **0–1** Low (0.7–3.2%) — home treatment possible
- **2** Moderate (13%) — hospital (short stay / supervised)
- **3–5** Severe (17–57%) — admit; 4–5 assess for ICU/high care

CRB-65 (NO UREA)

- **0** Low — home
- **1–2** Increased — consider hospital
- **3–4** High — urgent admission

SA STG

- Severe pneumonia = ≥2 of: confusion/↓LOC, RR ≥30, SBP <90, DBP <60, age >65 ‡ ✚
- Severe: oxygen to SpO_2 92%; ceftriaxone 1 g IV/IM single dose before referral ✚
- Refer if any: confusion/↓LOC, cyanosis, RR ≥30, SBP <90, DBP <60 ✚
- Hospital: severe pneumonia = cyanosis, confusion, hypotension or RR >30 ✚
- IV → oral switch when haemodynamically stable, RR <25 and T <37.8 °C ✚

PEARLS

- Score supports, not replaces, judgement — hypoxia, comorbidity (HIV, TB) and social factors also decide admission
- CRB-65 is for settings without urea results
- Underestimates severity in young patients with sepsis — check lactate/SpO_2

PHC STG & EML Primary Healthcare, 8th ed. 2024 (updated Sept 2026) — pp 17.25, 17.26 · **HOSP** STG & EML Hospital Level Adults, 6th ed. 2024 (updated Mar 2026) — pp 16.16 · **CURB65** Lim WS et al. Thorax 2003;58:377-82

✚ = SA STG / national guideline (pages listed above). Unmarked content: international source named above. ‡ = STG misprint or sources differ (see app). ANDH · curb65 · updated 2026-10-14